ICSR IN PHARMACOVIGILANCE

Step-by-step study notes • Key concepts • Interview revision

Note: These notes explain the standard ICSR workflow. They are a study guide based on the summary in this chat, not a verified transcript of the linked YouTube video. Specific reporting requirements and timelines depend on the applicable regulator, product type and local regulations.

1. Introduction to ICSR

Full form: Individual Case Safety Report (ICSR).

An ICSR is a structured report containing information about one or more suspected adverse events or adverse drug reactions associated with the use of a medicinal product. ICSRs are collected and assessed as part of pharmacovigilance to help monitor the safety of medicines and protect patients.

Purpose of an ICSR

• Collect and document suspected safety events associated with medicinal products.

• Support assessment of possible relationships between a product and an event.

• Identify potential safety signals and contribute to ongoing benefit–risk evaluation.

• Provide relevant safety information to regulators and other stakeholders.

2. Four Minimum Criteria of a Valid ICSR

A report generally needs all four elements below to qualify as a valid individual case for ICSR processing:

Minimum criterion	What it means
1. Identifiable patient	Enough information to distinguish the patient, such as initials, age, date of birth or another identifier.
2. Identifiable reporter	The source of the report can be identified, such as a healthcare professional or consumer.
3. Suspected medicinal product	At least one medicinal product is suspected of being associated with the event.
4. Suspected adverse event/reaction	An unwanted medical occurrence or suspected reaction is reported.

Remember: If one of the four minimum criteria is missing, the report may need clarification or follow-up before it can be treated as a valid ICSR. Do not invent missing information.

3. ICSR Processing – Step by Step

1. Case receipt

Receive the safety report from a source such as a spontaneous report, email, website, call centre, literature or a study. Record when and how it was received.

2. Case validation

Check whether the four minimum criteria are present. If important information is missing, request follow-up where appropriate.

3. Duplicate check

Search the safety database for an existing report that may describe the same patient, product and event. Manage potential duplicates according to the applicable procedure.

4. Data entry

Enter available information accurately, including patient and reporter details, suspected product, event description, dates, medical history and relevant test results.

5. Medical coding

Code reported medical signs, symptoms and diagnoses using the current MedDRA terminology and the organisation's coding conventions.

6. Drug coding

Identify and code the medicinal product using the relevant product dictionary or database. Capture product details such as name, dose, route and therapy dates when available.

7. Seriousness assessment

Assess whether the case meets regulatory seriousness criteria. Seriousness is based on outcome or medical significance, not simply the intensity of a symptom.

8. Expectedness/listedness assessment

Where required, compare the reaction with the applicable reference safety information to determine whether it is expected or listed.

9. Causality assessment

Evaluate the reported relationship between the product and the event using the applicable method. Consider timing, alternative causes, dechallenge and rechallenge information when available.

10. Quality review

Review the case for accuracy, consistency, completeness, correct coding and compliance with the applicable procedure.

11. Regulatory submission and follow-up

Submit the case to the appropriate recipient within the applicable reporting timeline. Track acknowledgements and obtain follow-up information when needed.

4. Important Pharmacovigilance Terminologies

Term	Meaning
Pharmacovigilance (PV)	Science and activities relating to the detection, assessment, understanding and prevention of adverse effects or other medicine-related problems.
AE	Adverse Event: any untoward medical occurrence in a patient or clinical-trial participant given a medicinal product; it does not necessarily have a causal relationship with treatment.
ADR	Adverse Drug Reaction: a harmful and unintended response for which a causal relationship with a medicinal product is at least a reasonable possibility, under the applicable definition.
SAE	Serious Adverse Event: an event meeting one or more seriousness criteria, such as death, life-threatening event, hospitalisation, disability, congenital anomaly or another medically important condition.
MedDRA	Medical Dictionary for Regulatory Activities; a standardised medical terminology used to code safety information.
E2B	ICH standard for the electronic transmission of individual case safety reports.
SUSAR	Suspected Unexpected Serious Adverse Reaction; a term used in clinical-trial safety reporting when the relevant criteria are met.
Follow-up	Additional information collected after the initial report to clarify or complete the case.

5. Seriousness vs Severity

Seriousness is a regulatory classification based on defined outcome criteria, such as hospitalisation or a life-threatening event. **Severity** describes the intensity of the event, such as mild, moderate or severe. A very intense symptom is not automatically a serious event; the seriousness criteria must be assessed.

6. Important Topics for Interviews

1. What is pharmacovigilance?
2. What is an ICSR?
3. What are the four minimum criteria of a valid ICSR?
4. What is the difference between an AE and an ADR?
5. What is the difference between seriousness and severity?
6. What is MedDRA and why is it used?
7. What is duplicate checking?
8. How is seriousness assessed?
9. What is causality assessment?
10. Why is follow-up information collected?
11. What is E2B?
12. Why are quality review and regulatory timelines important?

7. Seven-Day Learning Plan

Day	Topics to study
Day 1	Pharmacovigilance basics and ICSR introduction
Day 2	Four minimum criteria and case validity
Day 3	Case receipt, data entry and duplicate checking
Day 4	MedDRA and drug coding
Day 5	Seriousness and expectedness assessment
Day 6	Causality assessment and quality review
Day 7	Regulatory reporting, follow-up and interview revision

8. Quick Revision – Remember These First

• **First priority:** Learn the four minimum criteria of a valid ICSR.

• **Second priority:** Understand the complete case-processing workflow in the correct order.

• **Third priority:** Learn AE vs ADR and seriousness vs severity.

• **Fourth priority:** Understand MedDRA coding, duplicate checks and causality assessment.

• **Fifth priority:** Learn that reporting timelines and submission requirements depend on applicable regulations and case type.

Study tip: Learn the workflow as a sequence, then practise explaining each step in your own words. For real case processing, always follow current SOPs, product reference safety information and applicable regulatory guidance.